

# Headache - Male (Child)

Algorithm 1031 | Child (0-17 years) | Gender at birth: Male

## Definition

- Headache assessment decomposed from NHS.UK's published when-to-get-help guidance.
- Child patient.
- Male.

## Initial assessment

1. [OPEN\_TEXT] What is the person's exact age and sex, are they pregnant or within 6 weeks postpartum, when did the headache start and reach maximum severity, and is it first, worst, new, or different? Include trauma, fever, neurological/visual symptoms, anticoagulants, carbon monoxide, and safeguarding concerns.
2. [YES\_NO] Any recent head injury (within the last 3 months)?
3. [PAIN\_SCALE] How severe is the pain, 0-10?
4. [CAREGIVER\_OBSERVATION] What has a safe parent or caregiver observed about alertness, breathing, pain, swelling, fever, eating or drinking, sleep, play, and normal activity?

## Triage and redirect questions

### **1. Disposition level 100**

Has there been a seizure, numbness or weakness in the body or face, sudden extremely painful onset, difficulty speaking, balancing, walking, or remembering things, drowsiness or confusion, loss of vision, a non-fading rash, a very high temperature with a stiff neck or light sensitivity, or a recent major head injury with deterioration?

NHS.UK headache guidance lists these as Qatar 999/an emergency assessment service selected through the Qatar pathway approved by governance (GOVERNANCE\_REQUIRED) criteria - possible stroke, meningitis, or other serious cause.

### **1. Disposition level 70**

Did the headache occur after any head injury within the last 3 months, with none of the immediate life-threatening features above?

NHS.UK directs headache after head injury within 3 months to 999 or an emergency assessment service selected through the Qatar pathway approved by governance (GOVERNANCE\_REQUIRED). This branch preserves emergency assessment while avoiding an unsupported rule that every stable case requires a Qatar ambulance; exact local destination and transport remain GOVERNANCE\_REQUIRED.

### **1. Disposition level 70**

Are there vision or eye problems, is the headache triggered or worsened by coughing, sneezing, bending down, or exercise, is there vomiting, is a child's headache getting worse or waking them at night, or is there jaw pain when eating or a tender scalp?

NHS.UK guidance lists these as reasons to seek clinical review through a Qatar pathway approved by governance (GOVERNANCE\_REQUIRED) or get an urgent clinical review through a Qatar pathway approved by governance (GOVERNANCE\_REQUIRED).

### **1. Disposition level 15**

Is this a mild-to-moderate headache with none of the features above?

NHS.UK guidance recommends hydration, rest, and simple pain relief for uncomplicated headaches.

## **Advice**

### **Qatar emergency headache response**

- Call Qatar emergency services on 999 and do not drive for sudden maximal/severe headache, stroke signs, seizure, confusion, reduced consciousness, meningitis features, major head injury with deterioration, carbon-monoxide exposure, acute visual loss, or pregnancy/postpartum danger signs. Note the last-known-well time and do not give food, drink, aspirin, or unprescribed medicine when stroke, bleeding, or impaired swallowing is possible. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: worsening confusion; new weakness, numbness, speech, balance, or vision change; seizure, neck stiffness, rash, or reduced consciousness.

### **Emergency assessment after recent head injury**

- Arrange prompt in-person emergency assessment through the Qatar governance-approved pathway and do not self-drive. Call 999 immediately if severe or worsening headache, repeated vomiting, seizure, weakness/numbness, speech/balance/vision change, confusion, drowsiness, collapse, abnormal breathing, or other deterioration develops. Exact destination and transport for a stable patient remain GOVERNANCE\_REQUIRED.
- Call Back If: worsening headache or repeated vomiting; confusion, drowsiness, seizure, weakness, speech, balance, or vision change; collapse or abnormal breathing.

### **Urgent headache review**

- Arrange same-day or next-day medical review through the approved Qatar pathway, especially for vision changes or persistent vomiting.
- Call Back If: vision worsens; vomiting continues.

## ***Governed care for a familiar uncomplicated headache***

- Rest in a quiet environment, maintain usual fluids if not restricted, eat normally, and avoid alcohol and known triggers. Medicine choice/dose requires age, weight, pregnancy/breastfeeding, allergy, kidney/liver disease, ulcer/bleeding risk, anticoagulants, and current medicines under the approved Qatar pathway.
- Call Back If: headache persists, changes, or worsens; new fever, neck stiffness, vomiting, vision, speech, weakness, seizure, or pregnancy/postpartum symptoms.

## **First aid**

- Call Qatar emergency services on 999 and do not drive for sudden maximal/severe headache, stroke signs, seizure, confusion, reduced consciousness, meningitis features, major head injury with deterioration, carbon-monoxide exposure, acute visual loss, or pregnancy/postpartum danger signs. Note the last-known-well time and do not give food, drink, aspirin, or unprescribed medicine when stroke, bleeding, or impaired swallowing is possible. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

## **References**

- NHS.UK, "Headaches", <https://www.nhs.uk/conditions/headaches/> (page last reviewed 17 April 2024)

## **Provenance**

UAT DATA - NOT FOR REAL PATIENT CARE OR PRODUCTION. Qatar-localized structure-validation content generated deterministically from src/data/openSourceGuidelines/batch01.ts and cited public sources. It is not licensed STCC content. All disposition and redirect branches are non-telemedicine pending Qatar clinical and nursing governance approval. Generation does not import or deploy records. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

Manifest hash: 23887a9b59d7ccc7776d82ff394c23c61b2a2f3b580a5b3f6983af7e49e30bef | Status: BLOCKED\_BY\_RESEARCH\_GAP